

NOTICE OF INFORMED CONTEMPT

Hermann Goering assures cohorts they have immunity from the law, they were under orders. As you can see by the next photo that defense didn't work well for them.



When a **society** adopts a high-power biological tool,
it **has a choice** about what kind of relationship it wants with the public.

One is high-trust: institutions speak plainly, show their work, accept adversarial testing as normal, and treat consent as a real boundary rather than a public-relations inconvenience. In that world, large public-health campaigns can function because the system is stable enough that people can take a risk without feeling like they are being used.

The other is low-trust: institutions substitute pressure for proof, lean on credential authority instead of auditability, and treat dissent as a contamination event to be suppressed. In that world, it does not matter how benevolent the tool is supposed to be. The way it is managed turns it into a threat, because the public has no reliable way to distinguish “prophylactic” from “hostile,” “mistake” from “denial,” “safety signal” from “signal management.” Then it has gone from medicine to power.

The law understands this distinction better than our public discourse does.

The Biological Weapons Convention bans biological agents and toxins “of types and in quantities that have no justification for prophylactic, protective or other peaceful purposes,” and it also bans weapons and delivery systems designed to use them for hostile purposes. It **draws a bright line: peaceful purpose and justification on one side, hostile purpose and weaponization on the other.** [1]

U.S. law tracks the same boundary condition. 18 U.S.C. § 175 criminalizes possession or development of biological agents, toxins, or delivery systems “for use as a weapon,” and the statutory definition explicitly turns on purpose, including the absence of a prophylactic, protective, bona fide research, or other peaceful justification. [2]

In practice, **that line has consequences.** In a functioning system, the “**peaceful purpose**” side of the boundary is **not a slogan. It is a verifiable configuration:** transparent inputs, traceable manufacturing controls, intelligible risk disclosure, independent replication, open adverse-event adjudication, and a consent process that is robust enough to survive contact with incentives and fear.

If you want the public to treat a population-scale injection platform as a medical tool rather than a dual-use capability, you do not get there by demanding trust. You get there by earning it. Auditability is how you earn it.

That is the peace treaty.

Here are the terms, stated plainly.

First: bodily autonomy is not a negotiable resource. No mandates, no coerced compliance through employment, education, travel, or access to basic civil life. **If a product requires coercion to achieve uptake,** that is not a victory. **It is a signal, and it should be treated like one.**

Second: radical transparency is not optional. **The public is not obliged to accept “safe and effective” as a magic phrase.** Publish manufacturing quality metrics that matter to risk: lot-to-lot variation, impurity thresholds, process deviations, stability profiles, and the decision logs for how those were handled. **If you want this to feel like medicine, then treat it like medicine:** independent lot testing with publicly posted results should be normal, not scandalous.

Third: independent verification must be structurally protected. “Independent” does not mean “approved by the same bottlenecks.” It means third parties with no stake in procurement, no threat to licensing for dissent, and full access to the materials needed to test the claims. **A system that cannot tolerate adversarial testing is not a health system. It is an enforcement system.**

Fourth: adverse-event monitoring must be treated as a real sensor, not a reputational hazard. When a system is exposed to wide deployment, it will surface failure modes. **The correct response is legible repair:** clear thresholds for pausing, narrowing indications, updating warnings, and compensating harms without forcing people into years of attritional process.

Fifth: conflicts of interest must be disclosed at the level they actually operate. Not a token PDF. The real map: procurement incentives, revolving doors, liability shields, campaign finance adjacency, and the pressure gradient between regulators, manufacturers, and institutions that enforce compliance. **In any engineered system, you look at loads and incentives first. That is where failure modes breed.**

If these terms are met, you get a stable equilibrium. People can disagree, opt in or opt out, and still cooperate as neighbors, **because the system is not forcing them to pretend that trust is the same thing as evidence.**

That is peace.

If these terms are refused,
then the refusal itself communicates something important. It says: “We want compliance without audit.” And **that pushes the entire platform toward the bioweapon boundary conditions,** because the system is insisting on the very features that make hostile use possible: opacity, pressure, censorship, and unaccountable deployment.

At that point, the response cannot remain polite confusion. It becomes organized resistance.

Not violent resistance. Not reckless resistance. The kind of resistance appropriate to a society that still wants to be lawful: information pressure, reputational pressure, procurement pressure, and political pressure.

Call it fifth-generation warfare if you like. In plain English, it is **citizens learning to impose cost on institutions that treat them as a managed resource**.

That means relentless documentation. FOIA, discovery, and public archiving.

It means **refusing to let “memory-holing” win by default**. It means supporting journalists and researchers who publish primary documents rather than vibes. It means making contradictions legible with receipts, **so the cost of lying exceeds the benefit**.

It also means refusing the psychological trap where institutions bait dissenters into sounding unstable. If you want to win a memetic fight, you do not do it by screaming. You do it by being accurate, concise, and repeatable. You make the failure mode legible. You name the boundary. You show the incentive structure. Then you stop.

Some activists have started formalizing this posture, including proposals to treat certain platforms as bioweapons under domestic law, and they circulate template legislative language accordingly.[3]

A separate stream, framed as an Indigenous tribunal declaration, takes an even harder stance and explicitly labels the COVID injections “BIOLOGICAL AND TECHNOLOGICAL WEAPONS OF MASS DESTRUCTION.”[4]

These declarations exist; they are circulating; they are being treated by many as dispositive. The existence of that framing is itself a signal: baseline trust is gone, and the vacuum will be filled by the harshest available interpretations.

If institutions want to de-escalate, the path is not persuasion campaigns. It is not “debunking.” It is not moralizing. It is simple engineering: reduce opacity, reduce coercion, reduce conflicts of interest, increase auditability, and treat consent as a hard constraint.

If they will do that, then we can treat biomedical tools as medicine again.

If they will not, then they should not be surprised when **more and more people treat the entire apparatus as a hostile system, and respond accordingly**.

In a stable society, peace is not achieved by demanding that people sleep through risk. Peace is achieved by building systems where the public can stay awake, look directly at what is being done, and still choose cooperation when the incentives and the evidence actually support it.

Reading levels :

-Pureblood

-1 Dose

-2 Dose

-3+Dose

this scale accounts for the cognitive impairment observed among the “vaccinated”. [0]

It is meant as a guideline, not a rule due to each case being different (batch#, injection method, etc).

Consider anyone who has read the document in full and can tell you what it is as an honorary Pureblood.

Sources

[0] International Journal of Innovative Research in Medical Science 10(06):241-248 June 2025

https://www.researchgate.net/publication/392648624_Association_Between_COVID-19_Vaccination_and_Neuropsychiatric_Conditions

[1] International Committee of the Red Cross (ICRC). “Convention on the Prohibition of the Development, Production and Stockpiling of Bacteriological (Biological) and Toxin Weapons and on their Destruction (Biological Weapons Convention), Article I.” IHL Treaties Database. Retrieved January 16, 2026, from <https://ihl-databases.icrc.org/en/ihl-treaties/bwc-1972/article-1>

[2] United States House of Representatives, Office of the Law Revision Counsel. “18 U.S.C. § 175: Prohibitions with respect to biological weapons.” United States Code. Retrieved January 16, 2026, from <https://uscode.house.gov/view.xhtml?edition=prelim&num=0&req=granuleid%3AUSC-prelim-title18-section175>

[3] Sansone, Joseph. “I Am Going To War! Will You Join Me? 50 Different State Versions of the ‘Sansone mRNA Bioweapons Prohibition Act’.” Retrieved January 16, 2026, from <https://www.josephsansone.com/p/i-am-going-to-war-will-you-join-me>

[4] Alliance of Indigenous Nations (AIN). “AIN-Declaration-of-Bioweapons-8October2025-1.pdf.” Retrieved January 16, 2026, from <https://allianceofindigenousnations.org/wp-content/uploads/2025/10/AIN-Declaration-of-Bioweapons-8October2025-1.pdf>

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